

Intervention and the road to recovery

Everything you need to know about
early intervention and the
management of musculoskeletal
claims

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Takeaway Messages

- By the time you have been advised of a claim, it's **never** too early to intervene
- Claims management during **active** recovery should be light touch
- Claims management during **passive** recovery will benefit from intervention
- A **sustainable** GRTW can always benefit from further intervention

Intervention

- Can take many forms
 - Regular telephone contact
 - Funding for non-invasive treatment ie. physio, osteo, chiro
 - Vocational Rehabilitation/Case Management
 - Assessment ie. Independent Medical Evaluation, Functional Capacity Assessment, Ergonomic Assessment, Worksite Assessment, Job Demands Analysis, Vocational Assessment
- Early intervention
- Consider cost benefit
- Consider definition of disability

Active recovery

- Proactive treatment plan in place
- Specialists involved
- Policy holder participating in prescribed treatment ie. doing exercises, undergoing surgery
- Light touch – scheduled phone calls, linked to key treatment dates

Passive recovery

- No treatment plan in place
- Policy holder not motivated/participating in treatment
- Query treatment plan with GP
- If no treatment plan, may need further assessment (ie. IME, FCA) to help formulate
- Consider funding for non-invasive treatment
 - Consider motivation

Recovery – what now?

- Some people will RTW by themselves
- Some require assistance/intervention
- Consider likelihood of **sustainability** without assistance
- Consider cost benefit
- Consider definition of disability
- RTW intervention/assistance
 - Vocational Rehabilitation/Case Management
 - Functional Capacity Assessment
 - Ergonomic Assessment
 - Worksite Assessment
 - Job Demands Analysis

Vocational Rehabilitation/ Case Management

- Often allied health professional – PT, OT, psychologists
- Liaison between all parties – PH, employer, treating specialists, insurer
- Ideally affiliated with appropriate professional bodies/qualifications – Vocational Rehabilitation Association (vra-uk.org), Case Management Society UK (cmsuk.org)

Functional Capacity Assessment

- Standardized testing regime of a person's physical abilities
- Consider home based or clinic based
- Consider what you want out of the assessment and how you will use the information

Ergonomic Assessment

- Sedentary occupations
- Review workstation setup
- Consider how you will utilise recommendations – funding for equipment, Access to Work, employer co-contribution

Worksite Assessment

- Manual occupations
- Review duties – provide practical suggestions
- Consider how you will utilize recommendations – funding for equipment, Access to Work, employer co-contribution

Job Demands Analysis

- Thorough review of physical demands of role
 - Weights lifted
 - Repetitions
 - Timings
 - Heights
 - Manual handling technique
- Can be used to help formulate GRTW program
- Helpful for Occ Phys, GPs, OH, VR, CM

Occupational Health

- Often OH Nurses
- GRTW programmes tend to focus on hours worked, neglect duties
- For sedentary roles, not as big a concern – consider WFH
- For manual roles, duties are key – job demands analysis often helpful
- Sustainable GRTW schedule should consider all aspects

Return to work

- Once RTW commenced, **sustainability** most important
- Err to side of caution
- Consider that FRTW not always feasible – better to maintain at PRTW than to relapse
- If VR or CM involved, carry on until FRTW or peak PRTW
- If not, Claims Assessor to maintain regular contact

Alternative return to work

- Policy holder will not always be able to return to pre-injury duties
- FCA should help determine capacity for work
- Vocational assessment determine transferrable skills, identify suitable occupations
- Consider VR to help job seek

Questions

Thank you

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